

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Randomized, Open-label, Phase 3 Study of MK-2870 in Combination With Pembrolizumab Compared to Pembrolizumab Monotherapy in the First-line Treatment of Participants With Metastatic Non-small Cell Lung Cancer With PD-L1 TPS Greater Than or Equal to 50% (TroFuse-007)
Short Title:	Sacituzumab Tirumotecan (MK-2870) in Combination With Pembrolizumab Versus Pembrolizumab Alone in Metastatic Non-small Cell Lung Cancer (NSCLC) With Programmed Cell Death Ligand 1 (PD-L1) Tumor Proportion Score (TPS) ≥ 50% (MK-2870-007)
Protocol Number:	975840486149636360
NCT Number:	NCT06170788
Sponsor Name:	Merck
Investigational Product:	pembrolizumab
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Overall Survival (OS): OS is defined as the time from randomization to death from any cause.
Secondary Obj.:	1. Progression free survival (PFS) 2. Objective Response (OR) 3. Duration of Response (DOR)

2.2 Background & Rationale

The primary objective of the study is to compare sacituzumab tirumotecan combined with pembrolizumab to pembrolizumab alone with respect to overall survival (OS). The primary hypothesis is that the combination of sacituzumab tirumotecan and pembrolizumab is superior to pembrolizumab alone with respect to OS.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	614

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

* Histologically or cytologically confirmed diagnosis of squamous or nonsquamous NSCLC

* Confirmation that epidermal growth factor receptor- (EGFR-), anaplastic lymphoma kinase- (ALK-), or proto-oncogene

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

tyrosine-protein kinase ROS (ROS1-) directed therapy is not indicated as primary therapy

- * Provided tumor tissue that demonstrates programmed cell death ligand 1 (PD-L1) expression in ≥50% of tumor cells as assessed by an immunohistochemistry (IHC) central laboratory

- * An Eastern Cooperative Oncology Group (ECOG) performance status of 0 to 1 assessed within 7 days before randomization.

- * A life expectancy of at least 3 months.

- * Human immunodeficiency virus (HIV)-infected participants must have well controlled HIV on antiretroviral therapy (ART)

4.2 Exclusion Criteria

- * Diagnosis of small cell lung cancer or, for mixed tumors, presence of small cell elements.

- * Has Grade ≥2 peripheral neuropathy.

- * History of documented severe dry eye syndrome, severe Meibomian gland disease and/or blepharitis, or severe corneal disease that prevents/delays corneal healing.

- * Has active inflammatory bowel disease requiring immunosuppressive medication or previous clear history of inflammatory bowel disease (eg, Crohn's disease, ulcerative colitis, or chronic diarrhea).

- * Has uncontrolled, significant cardiovascular disease or cerebrovascular disease within the 6 months preceding study intervention.

- * Received prior systemic anticancer therapy for their metastatic NSCLC.

- * Received prior therapy with an anti-PD-1, anti-PD-L1, or anti-PD-L2 agent, or with an agent directed to another stimulatory or coinhibitory T-cell receptor Note: Prior treatment with an anti-PD-1, anti-PD- L1, or anti-PD-L2 agent in the neoadjuvant or adjuvant setting for nonmetastatic resectable NSCLC is allowed as long as therapy was completed at least 12 months before diagnosis of metastatic NSCLC.

- * Received prior systemic anticancer therapy including investigational agents within 4 weeks before randomization.

- * Received radiation therapy to the lung that is ≥30 Gy within 6 months of start of study intervention.

- * Received prior radiotherapy within 2 weeks of start of study intervention, or radiation-related toxicities, requiring corticosteroids.

- * Received a live or live-attenuated vaccine within 30 days before the first dose of study intervention. Administration of killed vaccines are allowed.

- * Diagnosis of immunodeficiency or is receiving chronic systemic steroid therapy

- * Known additional malignancy that is progressing or has required active treatment within the past 3 years.

- * Known active central nervous system (CNS) metastases and/or carcinomatous meningitis.

- * Known intolerance to sacituzumab tirumotecan or pembrolizumab and/or any of their excipients; for pembrolizumab, severe hypersensitivity (≥Grade 3) is exclusionary.

- * Known hypersensitivity to sacituzumab tirumotecan or other biologic therapy.

- * Active autoimmune disease that has required systemic treatment in the past 2 years.

- * History of (noninfectious) pneumonitis/interstitial lung disease (ILD) that required steroids or has current pneumonitis/ILD.

- * Active infection requiring systemic therapy

- * Concurrent active Hepatitis B and Hepatitis C virus infection.

- * Human immunodeficiency virus (HIV)-infected participants with a history of Kaposi's sarcoma and/or Multicentric

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

Castleman's Disease.

* History of allogeneic tissue/solid organ transplant.

* Requires treatment with a strong inhibitor or inducer of Cytochrome P450 3A4 (CYP3A4) at least 14 days before the first dose of study intervention and throughout the study.

11. Study Identifiers & Governance

Primary ID: 975840486149636360

Registry Number: NCT06170788

Sponsor: Merck

Study Title: Sacituzumab Tirumotecan (MK-2870) in Combination With Pembrolizumab Versus Pembrolizumab Alone in Metastatic Non-small Cell Lung Cancer (NSCLC) With Programmed Cell Death Ligand 1 (PD-L1) Tumor Proportion Score (TPS) ≥ 50% (MK-2870-007)

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____